

Patient Name:

Palliative Care Authorisation (Adults)
Cover Sheet
Sheet 1 of 4

COMMUNITY AUTHORISATION

Registered GP:	Tel:
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Allergies

St	E	Allergy or Sensitivity

This patient has been provided with medication to ensure their comfort if they are unable to take oral medications and/or in the period approaching end-of-life.		Initial/signature
1. At the time of this authorisation, the patient <u>was</u> felt to be in the final days/hours of life (GSF category D). <i>If these medications are not started within 14 days of the date of this authorisation (or if > 14 days has elapsed since these medications were last given), it should be confirmed that the patient is in the final days of life (see guidance below).</i>		
2. At the time of this authorisation, the patient <u>was not</u> felt to be in the final days of life (GSF category A-C). A. These medications have been prescribed in anticipation of a predicted deterioration ☐ B. Because the patient cannot swallow or absorb oral medications e.g. bowel obstruction ☐ <i>Before these medications are started (or if it is > 14 days since these medications were last given), it should be confirmed that medication detailed below is still appropriate for the patient's condition. Specifically – is correct opioid (and dose) prescribed based on current opioid use, is a syringe driver intended to be used in addition to current opioid or as a replacement, is the PRN dose appropriate?</i>		
Additional instructions/ clinical information (eg renal impairment; to include eGFR if available)		
<i>Detailed guidance to support safe prescribing, using this authorisation form, obtaining replenishment supplies and converting PRN doses for syringe driver is available at https://www.areaprescribingcommitteeleicesterleicestershireutland.nhs.uk/wp-content/uploads/2024/09/Guidance-doc-authorisations-FINAL-7.pdf</i>		
Signed:	Name and role:	
Contact number:	Date:	
Guidance <i>Unless prescribed for reason 2b, these medications should only be used to relieve symptoms in the last days of life. If the attending HCP is not confident to make this assessment or it has been >14 days since these medications were last given, discussion with another HCP should take place first and documented in the clinical notes. It is not necessary to re-write the authorisation as part of this discussion.</i>		

Patient Name:
Address:
DOB:
NHS no:

Palliative Care Authorisation (Adults)
PRN Medication

Sheet 2 of 4

COMMUNITY AUTHORISATION

Registered GP: _____ Tel: _____

Date* (each line used must be dated)	Approved Drug name	Indication	Dose (or dose range)	Revised Dose Range (cross out previous range)	Any additional instructions	Minimum Frequency	Max PRN dose in 24 hours	Prescriber's Signature (each line prescribed MUST be signed)	Name / Role / Contact No (need only once for each prescriber)
	Morphine	Pain / Breathlessness				1 hourly			
	Midazolam	Anxiety / agitation	2.5 to 5mg			1 hourly			
	Glycopyrronium	Troublesome respiratory secretions	200 micrograms			4 hourly	1.2mg (including syr. driver)		
	Levomepromazine	Nausea and vomiting	2.5 to 5mg			1 hourly	25mg		
	Levomepromazine	Agitation	6.25 to 12.5mg			1 hourly	50mg		
IF PATIENT IS ON AN ALTERNATIVE BASELINE OPIOID e.g. Oxycodone OR ANTIEMETIC PLEASE PRESCRIBE APPROPRIATE PRN BELOW (and delete above as appropriate)									

Patient Name:
Address:
DOB:
NHS no:

Palliative Care Authorisation (Adults) Syringe Driver Medication

COMMUNITY AUTHORISATION

Sheet 3 of 4

Registered GP: Tel:

Date	Approved drug name	Guidance on initial doses and indication	Dose per 24 hours	Prescriber's Signature (each line prescribed MUST be signed)	Revised Dose Range (cross out previous dose)	Revised range - Prescriber's signature with DATE	Any additional instructions (e.g. about range)** or diluent if not Water for Injections	Name / Role / Contact No (need only once for each prescriber)
	Morphine	5-20mg Pain/ Breathlessness						
	Midazolam	5-10mg Anxiety/ Agitation						
	Glycopyrronium	0.8-1.2mg Troublesome respiratory secretions						
	Levomepromazine	6.25 – 12.5mg Nausea & vomiting						
	Levomepromazine	6.25 – 12.5mg Agitation					Discussion with palliative care team before starting	
IF PATIENT IS ON AN ALTERNATIVE BASELINE OPIOID OR ANTIEMETIC PLEASE PRESCRIBE BELOW (and delete above as appropriate)								

Dose ranges ** – start at the lower end of the range and only increase if it becomes clear that the lower dose has been ineffective (e.g. the patient has required extra prn medication). Drugs are authorised to be mixed in a single syringe driver with water for injection. For compatibility advice (e.g. cyclizine or higher doses morphine/haloperidol) refer to: [The Management of Common Symptoms in the Last Few Days of Life](#).

Please use the table below to briefly document clinical discussion – full documentation should be saved on SystmOne / EMIS

Clinical discussions should consider the patient's overall clinical condition, any background opioids the patient is taking (formulation eg. patch / oral, and dose), if a syringe driver is intended to be used in addition to current medication or a replacement, and whether the PRN dose has been adjusted appropriately

Time and date	Brief details of conversation	Clinical discussion with (name, job title and organisation)	Your name and job title

A further clinical discussion may be required in the following situations:

- Uncontrolled symptoms
- Rapidly escalating medication requirements (eg. multiple PRN doses, opioid increase by greater than 50%)
- Unexplained deterioration (which may not be consistent with dying)
- Any other situation in which there is uncertainty about how best to proceed